LocSSIP 25 Care of the deceased patient in the Theatres department

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Care of the deceased refers to the physical care given by health care workers following death, which has traditionally been referred to as 'last offices'. For this procedure document, the term 'care after death' is more appropriate and reflects the multi–cultural society.
Any religious needs or cultural customs required for the care of the deceased or their relatives must be respected.
Guidelines for meeting the religious & spiritual needs of patients are available in Theatre 4's yellow LocSSIP file, the 611 file and the Team Leaders LocSSIP file.
Further information can be found at the BCU Spiritual Care and Chaplaincy page on Betsi Net (see link: BCU Spiritual Care and Chaplaincy - Home) or by contacting the 24-hour Chaplaincy Service via the main switchboard.
This LocSSIP is to be used in conjunction with
IPC03 Standard Infection Precautions Procedure
IPC32 - Infection Prevention Management of the Deceased Patient Procedure
IPC23 - Notifiable Diseases Procedure .pdf
IPC06 - Infection Prevention Hand Hygiene Procedure .pdf
Royal Marsden Hospital Last Offices guidelines.

1 NatSSIP 2 standards addressed within this procedural LocSSIP:

Organisational Standards-
Processes for safety- Governance
Processes for safety-Documentation
Processes for safety- Induction Processes
People for safety- Staff Training and Education
People for safety- Invasive Area Staffing and Resources
Performance for safety
Sequential Step Standard-
7 Sign out

2 Aims of the LocSSIP and key factors for consideration:

To continue care and maintain the dignity of the deceased patient.
To support religious and cultural beliefs held by the patient and their relatives.
To comply with legal and health and safety requirements.
To have a clear and safe process that maintains sensitivity, dignity and respect for the deceased patient and their families.

3 Responsibilities

3.1 Theatre Manager

Will ensure that the department and its staff are prepared for the appropriate management of deceased patients and their relatives to fulfil religious and cultural beliefs and to meet statutory health and safety legislation and legal requirements.

3.2 Theatre Team Leaders

Must ensure that their team members are aware of BCUHB and departmental standards of care required for deceased patients and their relatives.
Will ensure provision of items required for carrying out care after death.

3.3 Theatre Staff

Will ensure an appropriate standard of care and management is given to the deceased patient and their relatives.

4 Care after death (previously known as Last Offices)/Bereavement Box

4.1 Yellow Care after death/Bereavement Boxes containing equipment and relevant documentation can be found in the main theatre pack room at Ysbyty Gwynedd (on top of the shelves on the left).
4.2 When used, the boxes must be replenished as required.
Box 1 and 2 are complete and box 3 contains spare items for restocking.

5 Documentation

5.1 Death must be confirmed/diagnosed by a doctor and documented in the patient's medical notes.
The Doctor who confirmed the death must ensure their name and bleep number is clearly printed in the medical notes.
An entry is also to be made in the theatre information system (WPAS) and theatre care plan notes.

The proceedings prior to death are also to be recorded in the patient's medical notes and theatre care plan.
When family/significant others have been informed of the patient's death, this must be documented in the patient's notes where the date and time of death has been entered and also clearly documented on the notification slip attached to the patient's notes.
5.2 The Notification of Death form is to be completed by nursing staff, and placed onto the front of the patient's notes.

6 Communication

6.1 The person in charge of the theatre must immediately inform:-
The Theatre Department Manager or designated Deputy
The patient's ward / ITU or Emergency Department staff
The 019 bleep holder (for Main Theatres YG)
Out of hours-
The bleep 100 holder (Y.G.)
6.2 General porters are to be informed when the deceased patient is to be taken to the mortuary.
6.3 Medical staff must inform: -
Their senior colleagues.
The family of the patient's death -at the earliest opportunity. (This task may be undertaken by senior nursing staff).
His Majesty's Coroner (as all deaths in theatre require referral to the Coroner). It is the responsibility of the doctor to inform the family of this procedure and of the possibility of a post mortem examination to establish the cause of death.

7 Equipment management

7.1 All anaesthetic equipment used for that patient including drug syringes and ampoules should be kept and stored in a secure place, should further investigation as to cause of death be required (AAGBI 2005)
7.2 If there is suspicion of an equipment failure or hazard affecting the operating theatre, a decision may be made to take the theatre or anaesthetic machine out of commission until further notice.

8 Care of the Deceased Patient

Prior to personal care – please consider religious/cultural requirements (further advice is available from the Spiritual and Chaplaincy Service)

8.1 If there is any possibility of a police investigation regarding the death of the patient e.g., road traffic accident or industrial accident, or if the death is suspicious and/or unexplained, do not remove any endotracheal tubes, I.V. lines etc.
8.2 If there is any prospect of a police investigation into the death, advice must be sought from a senior member of theatre staff, and the decision documented, regarding washing the patient, as washing may remove important forensic evidence. Staff MUST be certain prior to any washing or further procedures are undertaken to the body, that there will be no police involvement in the case.
8.3 Carry out last offices as per Royal Marsden Last Offices guidelines (copy in yellow box). Do not shave the deceased patient as this cause bruising and marking which can appear days later, (this can be discussed with the family where possible). Usually, the funeral director will do this. Be aware that some faith groups prohibit shaving.
8.4 Infection control standards to be maintained during care of the deceased patient as per departmental ODSOP 28 Theatres Infection Prevention.pdf.
When the patient is known or suspected to be an infection risk, please see Appendix 1- Notifiable diseases and Appendix 2- Transmission based precautions to key infections in the deceased.
In normal working hours the Infection Prevention team can be contacted, or when out of hours the on call medical Microbiologist can be contacted for advice on safe transportation of the deceased patient from theatres.
In the case of a patient with suspected Viral Haemorrhagic Fever please refer to IPC 36 Protocol for Haemorrhagic Fevers (VHF) Section 8 available on BetsiNet.
8.5 For bodies infected with pathogens that may pose a serious risk to health, a completed risk category form must be attached to the patient's medical notes and a label identifying the type of infection risk in the clear pocket of the cadaver bag. See Appendix 3.
Please see Appendix 2 to ascertain if the deceased person must be placed in a body bag.
If the above situation occurs or when there is likely to be leakage of body fluids during transportation of the body, a body bag should be used.
Request for a bag can be made out of hours via the bleep 100 holder.
If the deceased patient is placed in a body bag, they must NOT be wrapped in a sheet.
8.6 Tracheostomy and endo-tracheal tubes are to remain in situ but *may* be removed if the doctor who confirms death is able to issue a Medical Certificate of Cause of Death- i.e., the death is due to natural causes.
If there is any cause for concern regarding the placement of the endotracheal tube, its position should be confirmed and recorded by an independent anaesthetist.

If a patient has been intubated for a period of time. e.g., on ITU or there are no prospects of legal proceedings, an anaesthetist may allow the removal of tubes with documentation of the decision.

8.7 All other drainage tubes including naso-gastric tubes, lines, surgical drains, chest drains or catheters etc. should be left undisturbed or managed as above.

If catheters or cannulas are to remain in situ, they must be closed with a spigot.

Stoma bags should be renewed.

Apply a clean waterproof dressing to any wound(s).

8.8 Referring to the patient's case notes, the patient should be correctly identified by placing an identity bracelet on one wrist and one ankle on different sides of the body.

If the deceased has a missing limb the identity bracelets should be placed on the same side of the body.

A second independent person checker is required to verify these details, one of which MUST be a Registered Nurse.

8.9 Bar coded electronic identity bracelets may be used.

When non electronic identity bracelets are used, details must be clearly written in capital letters in black pen. Details to include- full name: address: hospital D number: date of birth.

8.10 Care after death (Last Offices) are to be performed as per Royal Marsden Hospital guidelines.

Please refer to the checklist in the yellow bereavement box.

8.11 Tape a completed mortuary card to the shroud at chest level.

Wrap the deceased patient in a sheet. (If the deceased patient is placed in a body bag, they must NOT be wrapped in a sheet.)

Tape a mortuary label to the outer sheet bearing the patient's full name, address and D Number or when a body bag is used attach this label to the outside of the bag.

A second independent person checker is required to verify these details, one of which MUST be a Registered Nurse.

Also record on the label if the patient has the following: -

Cardiac Pacemakers

Joint replacements

Implantable Cardioverter Defibrillators

or has been injected with a Radioactive Isotope

8.12 Once care after death is completed the nurse in charge will arrange, as soon as possible, transfer to the mortuary by calling for a general porter (phone 851204).

The verbal code BTM (body to mortuary) may be used if upset could be caused by using entire phrase.

The nurse must ensure the correct identity of the body before allowing transfer to the mortuary.
8.13 General Portering staff will bring the mortuary trolley to theatre recovery/reception, via the main corridor.
Consideration must be given as to the activity in main reception/recovery prior to calling for the porter as patients may need to be delayed until transfer of the deceased patient from the theatre department is completed.
8.14 The patient's case notes, with death notification slip attached, and any relevant documents are sent with the body to the mortuary, in a sealed envelope.

9 Management of the patient's personal belongings

9.1 All belongings are to be identified, checked and listed in the Patient's Property book and the entry signed for by two members of nursing staff.
9.2 Jewellery should not be left on the patient unless otherwise instructed by a relative.
If jewellery such as a wedding ring or body piercing is left in situ, it is to be secured with tape, documented and signed for, in the nursing notes and on the mortuary card and Patient Property Book.
9.3 Any other valuables such as watches, bank cards, cash, mobile phones, keys etc. are to be sent to general office with completed copies of the Property Book sheet.
9.4 Out of hours- Contact general porters and arrange to meet at general office where the hospital night safe is located, to deposit valuables and copies of documentation.
9.5 Clothing is sent with the patient to the mortuary, in a labelled bag.
Clothing may be passed to next of kin when its receipt is acknowledged and signed for, in the Property Book.

10 Care of the Organ Donor Patient

10.1 An organ donation reference box is available in theatre 4.
For donor referrals and advice phone the 24-hr pager 03000 20 30 40 and ask that the on-call Specialist Nurse for Organ Donation rings theatre – always leave your name, the hospital name and the full contact number of the hospital including the extension you wish to be contacted on.
10.2 The In-house Specialist Nurse for Organ Donation is Abi Roberts 03000 850865/ 07889304437.
The In-house Clinical Lead for Organ Donation is Pierre Peyrasse.
The identified link nurse within the YG department is Caroline Evans.

The general principles of care as stated in the above, will apply.
10.3 The Specialist Nurse for Organ Donation will work with theatre staff to ensure all local policies/procedures and guidelines for care of the deceased patient are followed.

11 Information and support to relatives

11.1 When the death of a patient has occurred in the theatres department, the patient's family may wish to have a viewing.
11.2 On Monday to Friday, up to 3 pm, the patient will be transferred to the mortuary by General Porters, and a viewing can be arranged with the Mortuary staff. Appointments can be made by contacting 03000 850865.
These viewing will take place in the bereavement viewing room. One viewing can be arranged per family, and a viewing will be for 30 minutes.
11.3 Outside these hours, it may be possible for a viewing to take place in the Theatres department.
Sensitivity is required as this may be very emotional and/or distressing for the patient's family.
An appropriate location will be dependent on the location of other patients in the theatre department.
Ideally a viewing can take place in an isolated corner of Recovery, or in the PACU unit.
While it is preferrable to use Recovery or PACU, when these locations are busy, it may be possible to use an anaesthetic room. Consider using theatre 6 or 7 anaesthetic rooms due to their location.
It is not appropriate for a viewing to take place in the theatre or anaesthetic room which was the location of the patient's death.
11.4 Consider the appearance of the patient, ensuring they appear as presentable as possible, whilst still adhering to the requirements e.g., requirement to leave tubes in situ, possible requirement not to wash with police involvement. If there is police involvement follow their guidance.
If possible, the patient should be on a bed, rather than a trolley.
The patient's family will be accompanied at all times in the department.
Recovery staff to chaperone the family when possible.
11.5 If the family are waiting for a viewing in the department, they should be located in the surgeons' room, to allow privacy and dignity.

12 Support of staff

12.1 Following the death of a patient, a team debrief must be held during which a decision is to be made following principles of good practice, as to whether to cancel patients remaining on that list.
The decision to cancel patients must be clearly and promptly communicated to all relevant persons, wards and departments as per ODSOP 18 Operating list management.pdf.
12.2 Names of members of staff involved are to be noted by the team leader / 611 or 019 bleep holder and given to the theatre manager to enable further theatre team 'de-briefing' sessions to be held as necessary.
The Chaplain may be contacted if required.
12.3 Any concerns regarding death, or the systems of support in place for staff, must be reported to the bereavement nurse as soon as possible.
Bereavement support is available from the Bereavement Office- Ysbyty Gwynedd 03000 850865.
Emotional and Counselling Support is also available through the Occupational Health and Wellbeing Service.
Contact 03000854599.

13 Procedural Response to Unexpected Death in Children- PRUDIC

13.1 If a young person dies unexpectedly whilst an inpatient on an adult based ward or environment then the PRUDIC process will need to be followed diligently.
This procedural response will be followed when a decision has been made by the police that the death of a child is unexpected and the Procedural Response to Unexpected Deaths in Childhood (PRUDiC) is to be initiated (PRUDiC (Procedural Response to Unexpected Deaths in Childhood) - Public Health Wales (nhs.wales))
13.2 This procedure is available on the Safeguarding children web page on the Intranet. The on-call Paediatrician must be informed as per PRUDIC procedure and they will take a lead if this situation arises.
13.3 The Safeguarding Liaison Nurse in the DGH will need to be informed promptly of this occurrence, as they will provide support to the staff and assist them in the management of the process.
13.4 The person in charge of the department – Bleep 019 (Bleep 611 out of hours) will liaise with the on-call Paediatrician and the children's ward to ensure the following are informed -
HM Coroner
Police

General Practitioner
Health Board Head of Safeguarding
Children's Social Care

Please see
'Unexpected Death as a Hospital Inpatient' page 19 of PRUDIC Guidelines
and
BCUHB Standard Operating Procedure– Procedural Response to the Unexpected Deaths in Childhood (PRUDIC) Governance Framework
13.5 Whenever a child dies unexpectedly health professionals will consider the need to report as a Serious Incident or No Surprises Incident to Welsh Government in accordance with the 'Putting Things Right' Guidance.

14 Incident documentation and audit processes

14.1 Any patient safety incidents recorded using the Datix system will be investigated as appropriate.
14.2 An investigation will be completed for any incidents to understand all aspects and prevent reoccurrence.
14.3 Monthly Datix data is displayed on the information board in Main Theatres.
14.4 Regular LocSSIP audits are performed to ensure compliance of standards.
14.5 Relevant Incidents discussed in the Quality and Safety monthly meetings.
14.6 Lessons learnt for incident reviews cascaded though the department as required.
14.7 Any incidents which affect patient experience or patient care will be investigated and Duty of Candour applied where appropriate.

15 References

IPC03 Standard Infection Precautions Procedure
IPC32 - Infection Prevention Management of the Deceased Patient Procedure
IPC23 - Notifiable Diseases Procedure .pdf
IPC06 - Infection Prevention Hand Hygiene Procedure .pdf
PRUDiC (Procedural Response to Unexpected Deaths in Childhood)-Public Health Wales (nhs2023)
Standards & Recommendations for Safe Peri-operative Practice AfPP 2022
Royal Marsden Hospital Last Offices Guidelines-
Last Offices - Royal Marsden Manual accessed 01/04/2026
AAGBI. Catastrophes in Anaesthetic Practice. Dealing with the Aftermath- September 2005
BCUHB SOP – Procedural Response to the Unexpected Deaths in Childhood (PRUDIC) Governance Framework 2021
F01 – Patients Property and Monies – Wards and Departments
BCUHB Organ & Tissue Donation Policy. V2
Protocol for the Identification and Management of Patients with Suspected & Confirmed Viral Haemorrhagic Fevers (VHF).

Version	Date	Author	Reason
1	12/2000	Toni Lowe	
1	03/2003	Toni Lowe	Reviewed
1	10/2005	Toni Lowe	Reviewed
1.1	10/2008	Toni Lowe	Updated
1.1	10/2009	Toni Lowe	Reviewed
1.1	06/2013	Toni Lowe	Reviewed
1.1	05/2015	Toni Lowe	Reviewed
1.2	02/2016	Toni Lowe	Addendum added
1.2	04/2019	Toni Lowe	Reviewed
1.3	05/2022	Toni Lowe/Shan Roberts	New Format &Reviewed
1.4	06/2024	Shan Roberts	Reviewed & NatSSIP 2 standards updated
1.5	07/2025	Shan Roberts	Reviewed and section 10 updated.
1.6	04/2026	Shan Roberts	Reviewed and shaving guidance updated.

Appendix 1 – Notifiable diseases

Notifiable diseases

Disease	Whether likely to be routine or urgent
Acute encephalitis	Routine
Acute flaccid paralysis or acute flaccid myelitis (AFP or AFM)	**Urgent**
Acute infectious hepatitis (A/B/C)	**Urgent**
Acute meningitis	**Urgent**
Acute poliomyelitis	**Urgent**
Anthrax	**Urgent**
Botulism	**Urgent**
Brucellosis	Routine. **Urgent** if acquired in UK
Chickenpox (varicella)	Routine
Cholera	**Urgent**
Congenital syphilis	Routine
COVID-19	Routine
Creutzfeldt-Jakob disease (CJD)	Routine
Diphtheria	**Urgent**
Disseminated gonococcal infection (DGI)	Routine
Enteric fever (typhoid or paratyphoid fever)	**Urgent**
Food poisoning	Routine. **Urgent** if part of a cluster or outbreak
Haemolytic uraemic syndrome (HUS)	**Urgent**
Infectious bloody diarrhoea	**Urgent**
Influenza of zoonotic origin	**Urgent**
Invasive group A streptococcal disease	**Urgent**
Legionnaires' disease	**Urgent**
Leprosy	Routine

Disease	Whether likely to be routine or urgent
Malaria	Routine. **Urgent** if acquired in UK
Measles	**Urgent**
Meningococcal septicaemia	**Urgent**
Middle East respiratory syndrome (MERS)	**Urgent**
Mpox (previously known as monkeypox)	**Urgent**
Mumps	Routine
Neonatal herpes	Routine
Plague	**Urgent**
Rabies	**Urgent**
Rubella	Routine
Severe Acute Respiratory Syndrome (SARS)	**Urgent**
Scarlet fever	Routine
Smallpox	**Urgent**
Tetanus	Routine. **Urgent** if associated with injecting drug use
Tuberculosis Report suspected cases of TB to the National TB Surveillance System	Routine. **Urgent** if healthcare worker or suspected cluster or multi-drug resistant
Typhus	Routine
Viral haemorrhagic fever (VHF)	**Urgent**
Whooping cough	**Urgent** if diagnosed in acute phase. Routine in later diagnosis
Yellow fever	Routine. **Urgent** if acquired in UK

Sourced: UKSHA (2025)

Appendix 2 Transmission based precautions to key infections in the deceased

The causative agents for the key infections listed below have been arranged according to the most likely route of transmission, taking account of the activity when handling the deceased, e.g., through post-mortem and embalming.

Infection	Causative agent	Hazard group	Is a body bag needed[1]?	Can the body be viewed?	Can post-mortem be carried out?	Can hygienic treatment be carried out?	Can embalming be carried out?
Airborne Small particles that can remain airborne with potential for transmission by inhalation							
Tuberculosis	*Mycobacterium tuberculosis*	3	Yes	Yes[2]	Yes[3]	Yes	Yes[3]
Middle East respiratory syndrome (MERS)	MERS coronavirus	3	Yes	Yes	Yes[3]	Yes	Yes[3]
Severe acute respiratory syndromes (SARS)	eg SARS coronavirus	3	Yes	Yes	Yes[3]	Yes	Yes[3]
Droplet Large particles that do not remain airborne for very long and do not travel far from source with potential for transmission via mucocutaneous routes (ie mouth, nose or eyes)							
Meningococcal septicaemia (meningitis)	*Neisseria meningitidis*	2	No	Yes	Yes[5]	Yes	Yes[5]
Flu (animal origin)	eg H5 and H7 influenza viruses	3	No	Yes	Yes[5]	Yes	Yes[5]
Diphtheria	*Corynebacterium diptheriae*	2	No	Yes	Yes	Yes	Yes
Contact Either direct via hands of employees, or indirect via equipment and other contaminated articles where transmission is primarily via an ingestion route							
Invasive streptococcal infection	*Streptococcus pyogenes* (Group A)	2	Yes	Yes	Yes[5]	No	No
Dysentery (shigellosis)	*Shigella dysenteriae (type 1)*	3	No[6]	Yes	Yes	Yes	Yes
Hepatitis A	Hepatitis A virus	2	No[6]	Yes	Yes	Yes	Yes
Hepatitis E	Hepatitis E virus	3	No[6]	Yes	Yes	Yes	Yes
Enteric fever (typhoid/ paratyphoid)	*Salmonella typhi/ paratyphi*	3	No[6]	Yes	Yes	Yes	Yes
Brucellosis	*Brucella melitensis*	3	No	Yes	Yes[4]	Yes	Yes[4]
Haemolytic uraemic syndrome	Verocytotoxin/ shiga toxin-producing *E.coli* (eg O157: H7)	3	No[6]	Yes	Yes[4]	Yes	Yes[4]

Infection	Causative agent	Hazard group	Is a body bag needed[1]?	Can the body be viewed?	Can post-mortem be carried out?	Can hygienic treatment be carried out?	Can embalming be carried out?
Contact Either direct or indirect contact with blood/other blood containing body fluids via a skin-penetrating injury or via broken skin and through splashes of blood/other blood containing body fluids to eyes, nose and mouth							
Acquired immune deficiency syndrome (AIDS)-related illness	Human immunodeficiency virus	3	No	Yes	Yes[7]	Yes	Yes[7]
Anthrax	*Bacillus anthracis*	3	Yes	No	Yes[8]	No	No
Hepatitis B, D and C	Hepatitis B, D and C viruses	3	No	Yes	Yes[7]	Yes	Yes[7]
Rabies	Lyssaviruses	3	No	Yes	No	No	No
Viral haemorrhagic fevers	Specifically Lassa fever, Ebola, Marburg, Crimean-Congo haemorrhagic fever viruses	4	Yes[9]	No	No	No	No
Contact Either direct or indirect contact with body fluids (eg brain and other neurological tissue) via a skin-penetrating injury or via broken skin							
Transmissible spongiform encephalopathies (eg CJD)	Various prions	3	Yes	Yes	Yes[10]	Yes	No

Key

Red Minimise procedures or handling of the deceased

Yellow TBPs are necessary when carrying out procedures or handling the deceased

The highlighted areas indicate an increased level of risk associated with the infection to workers (with areas in red posing increased risk) and therefore require additional control measures when handling the deceased.

Notes

1 It is advised that a body bag is used for the deceased in all cases where there is, or is likely to be, leakage of body fluids.

2 With appropriate measures to deal with potential release of aerosols (eg place cloth or mask over mouth when moving the deceased).

3 With appropriate measures to deal with aerosol-generating procedures.

4 With measures to minimise environmental contamination (because of low infectious dose; ie the amount of pathogen or number of bacteria required to cause an infection is low).

5 With appropriate measures to prevent exposure of mucosal surfaces (eg a physical barrier to protect eyes, mouth and nose, such as a facemask or visor).

6 Although illness may have increased likelihood of leakage of body fluids.

7 With appropriate robust measures for the use of sharps (eg minimise use or use safer sharps devices).

8 Before undertaking a procedure, the rationale for a post-mortem should be carefully considered where anthrax infection is suspected, particularly where examination may increase the potential for aerosol generation.

9 With double body bag.

10 With appropriate measures to minimise percutaneous injury and contamination of work area, and to help with decontamination (eg high-level sharps control or dedicated equipment).

Source: HSE (2018)

Appendix 3 Hazard Notification Sheet

This form must accompany deceased patients with suspected or known infection being transferred from the ward to the mortuary or the funeral directors.

1	**Name of deceased**			
2	**Date and time of death**			
3	**Source (hospital, ward or other)**			
4 Infection risk from the deceased[1]				
4a	Does the deceased present an infection risk? (Ring as appropriate)			
	Yes	Suspected	None suspected	
4b	If yes, what are the likely routes of transmission? (Ring all that apply)[2]			
	Airborne	Droplet	Contact	
4c	Infection (if permitted to disclose)[3]			
4d	Provide any relevant information to enable the deceased to be handled safely[4]			
5 Condition of the deceased[5]				
5a	Is the deceased leaking body fluids? Please provide details			
5b	Have accessories that present a risk of sharps injury been removed?			
5c	If yes, have the puncture points been covered or sealed?			
5d	If no, please provide details and location			
5e	Does the deceased have an implantable device? (Ring as appropriate)			
	No	Yes and switched off	Yes but not switched off	
5f	If yes, please provide details and location			
5g	Was the deceased receiving radiotherapy? (If yes, please provide details)			
6	**Signed**[6]			
	Print name			
	Institution			

Please retain original copy in medical case notes. Two copies should be placed in a sealed envelope marked for attention of 'Mortuary' or 'Undertaker' (whichever is appropriate). The envelope must be marked 'Confidential'.